

Can We Broaden Our Reach?

Strengths-Based Approaches to Disability

This chapter addresses the question of why children with disabilities continue to be held to low expectations and to be educated in segregated settings. To understand how to answer the central question of Chapter 1, What is special education?, in an inclusive and strengths-based era, we went back to Public Law 94-142, the 1975 Education for All Handicapped Children Act (EHA), which is the precursor to the 2004 Individuals With Disabilities Education Act (IDEA). Likewise, in understanding why children with disabilities continue to be segregated and held to low expectations, we again begin with the EHA.

The question of who we teach as it pertains to special education dates back to the days following the passage of the EHA. Lance (1976) published an article asking who the "all" in the Education for All Handicapped Children Act were. In that article, published only one year after EHA was signed into legislation and two years before it was required to be fully implemented, Lance identified the reason for the establishment of special education as "the recognition of the need to provide different treatments to individuals with obviously differing capacities for benefiting from the traditional educational practices" (p. 68). In his discussion, Lance cited a 1974 resolution from the Council for Exceptional Children as stating that children who did not have such access were "children for whom educators have responsibilities to assist in prevention of further injury and to provide programs to remediate the damage that has occurred" (p. 72).

Wehmeyer (2019) noted that this description was typical of how the purpose of the EHA and special education was discussed at the time the law was

passed and that the bulk of these discussions used terms like those used by Lance and CEC: *treatment, prevention, damage, and reduction of injury*. All of these are medical terms. The purpose of EHA was to *treat* children with disabilities, who were understood solely in medical terms at the time: to *prevent* future *impairment* and *disability* and to *reduce injury, damage, and harm* from the disability. These purposes were consistent with how disability was understood at that time, and in this chapter we focus on the issue of how disability was and is understood and how changes to such understandings benefit inclusive and personalized education.

Understanding Disability

Let us first establish that how we understand disability has a direct and causal impact on how we design interventions to support people whom we determine have that disability. This is so much the case that we often name or describe the eras of disability history in the dual terms associated with how disability was understood and the associated treatment paradigm. Thus, Thompson et al. (2017) identified the two overarching historic paradigms in disability services as the medical/institutional paradigm and the normalization/community services paradigm. Both, however, were predicated on a biomedical model of disability.

Rather than rely solely on present levels of achievement and the child's disability, the program must take into consideration the child's potential for growth and the child's unique circumstances.

Biomedical Models of Disability

The task of defining disability for the first century of the history of the field of disability services was left to the professionals who had established the field and created the first systems of services. These were medical professionals who adhered to biomedical models of disability. Simply speaking, such models view disability within the context of long-term or chronic health conditions and as an internal pathology, resulting from disease or impairment (Wehmeyer, 2013). The interventions or treatments derived from such an understanding are, not surprisingly, medically oriented. The early history of disability is one of the development and expansion of hospitals and institutions implementing treatments

designed by medical professionals using medical models and medical practices (Smith & Wehmeyer, 2012). While originally intended to be habilitative, these services became increasingly segregated, involving larger and larger congregate settings with the purpose of, at their best, protection and care and, at their worst, isolation and restriction.

By the mid-20th century, the professionalization of the disability field had proliferated to include psychologists, therapists of all kinds, educators, support staff, and others, all of whom operated within biomedical understandings of disability. Eventually, the abuses and flaws of these historic institutional paradigms became so obvious that family members (mostly parents, along with some professionals) began to demand something different (Abeson & Davis, 2000). Buoyed by the post-World War II zeitgeist that anything was possible, these advocates created a community-based system of services that, by the end of the century, had radically changed where people with disability lived. They did this through the courts, through legislation, and through the creation of innovative supports. They also created the opportunity for children with disabilities to attend school. It was this so-called parent movement that, in essence, established what we now call IDEA.

And yet, despite all the progress made during this post-World War II era, how disability was understood fundamentally did not change. It continued to be understood within the biomedical model in which disability was conceptualized in terms of pathology, defects, and deficits. The newly established special education system thus inherited an understanding of disability as pathology and deficit and created an educational version of the medical model of services: separate and segregated classrooms and buildings where practitioners emphasized (back to Lance, 1976) treatment, prevention, and reduction of damage and injury.

Social Models of Disability

There was movement afoot, however, to change how disability was understood, and it was initiated within the medical system itself. Medical professionals concluded that there were limitations to conceptualizing long-term or chronic health care issues (including disability) within a disease and deficit model. In 1980, the World Health Organization (WHO) introduced the International Classification of Impairments, Disabilities, and Handicaps (ICIDH). Prior to this, the WHO included disability (across various categories) as a disease or a disorder, but the

ICIDH broke away from this. The ICIDH was intended not as a classification of diseases or disorders but as a means to classify the consequences of disease, injuries, and other disorders and of their implications for the lives of the people affected. For the first time, at least at a large-scale, systematic level, disability was recognized not simply as a function of health problems but instead as a function of interactions among and relationships between health-related issues, the context in which the person wants to function, and the characteristics of the person.

In 2001, the WHO took these ideas a step further, introducing the International Classification of Functioning, Disability, and Health (ICF). The ICF extended this understanding of disability as a function of the interrelationships between and among health, environmental, and personal factors and conceptualized disability as a part of,

and not apart from, typical human functioning. Models of disability such as the ICF

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are identified as social-ecological or person-environment fit models because they shift the locus of disability from within the person (as was the case in the biomedical model) to the gap between the person's abilities and capacities and the demands of the environment. That is, disability resides only in the gap between what people can do and what they want to do (Buntinx, 2013).

A second, and perhaps more important, movement was emerging at the same time: the disability rights movement. The parent era established critical civil rights legislation, including the right to a free, appropriate public education. At the same time, people with disabilities were coming together to create a disability rights and self-advocacy movement (Driedger, 1989). One emphasis within this rights-based self-advocacy movement was the "advancement of a positive disability identity and culture" (Caldwell, 2011). Longmore (2003) called the establishment of disability identity and culture the second phase of the disability rights movement, and Longmore and Umansky (2001) argued that,

while public policy has sought to fashion disability as a generic category and attempted to impose that classification of people with an assortment of conditions, disability has never been a monolithic grouping. There has always been a variety of disability experiences. [These] experiences of cultural devaluation and socially imposed restriction, of personal and

collective struggles for self-definition and self-determination—recur across the various disability groups and throughout their personal histories. (p. 4)

Leaders in the disability rights and self-advocacy movement emphasize that, by embracing disability identity, they recapture their personhood and lay claim to social justice, full citizenship, and participation. What emerged from this movement was what is referred to as a social model of disability, which views disability as arising "from the discrimination and disadvantage individuals experience in relation to others because of their particular differences and characteristics" (Bach, 2017, p. 40). Within a social model,

the unit of analysis shifts from the individual to the legal, social, economic, and political structures that calculate value and status on the basis of difference. Informed by principles of human rights, and an equality of outcomes that takes account of differences, the social model does not reject bio-medical knowledge of impairments and research on individual rehabilitation. Rather, it celebrates impairment as part of the human condition, and looks at achieving equity for people with impairments in terms of the social, cultural, and political contexts. (Bach, 2017, p. 40)

Social-ecological or person-environment fit models of disability are situated within the broader context of social models. There is some tension among advocates who emphasize the identity aspects of disability and ascribe to a pure social model and the social-ecological models that evolved from the medical system. For one, the latter still include medical and health formulations in the understanding of disability. However, while these differences are important, and we support the movement to embrace disability as identity, a social-ecological model lends itself better to education and the development of interventions and supports. And, among the many points of agreement between social and social-ecological models, a critically important one is that we cannot understand disability without taking into account people's strengths and abilities and the contexts in which people live, learn, work, and play.

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"Having" a Disability: An Outdated Phrase

In Chapter 1 we briefly discussed how IDEA identifies students who are eligible for special education services. Basically, eligibility requires that students meet the definition of one of the disability categorical areas and need specially designed instruction to be successful. We want to use this requirement to briefly consider the notion of a person "having" a disability. When disability was understood within a biomedical model, it was logical to refer to someone as having a disability, since it was understood that the locus of the disability was within the person. But when we view disability as a function of the environment and context, as does the social model, or as the interaction between personal capacity and the demands of the environment, as does the person-environment fit model, the locus of the disability lies outside of the person—the person does not *have* a disability. In the social model, disability is something imposed on people by environments, contexts, and systems. In social-ecological or person-environment fit models, disability resides in the gap between what someone can do and what someone wants to do.

In the next section, we discuss implications for these changing understandings of disability for inclusive education. But before we enumerate some of these implications, we want to note that, as long as our society continues to see people as "having" disability, then we in essence perpetuate biomedical notions of disease, pathology, and deficits. In a similar manner, if we continue to adhere to IDEA's process of eligibility for specially designed instruction because of "diagnosis" of a disability, we in turn perpetuate a system of segregation and discrimination. The solution is to adopt educational practices that focus on all children, are strengths based, emphasize universal design, focus on schoolwide approaches that provide high-quality instruction for all students, emphasize student agency and self-determined learning, abandon the notion of average as useful, and ensure personalized education for all children. These are the focal points for the next section.

Implications for Inclusive Education

To consider how changing understandings of disability impact inclusive education, we focus on social-ecological or, as we prefer, person-environment fit models of disability. These models begin with a person's strengths and capacities and then consider what is needed to function successfully in the environments and contexts in which that person wants to function. As noted, while we understand

and fully endorse disability as identity and pure social models of disability, the fact is that education is an intervention-oriented enterprise. Our role as educators is to improve student knowledge and skills so that students can be successful. As such, the person-environment fit model provides multiple ways we can proceed to design such interventions: we can provide learning opportunities that enable students to enhance their skills and knowledge needed to function successfully in the preferred context; we can modify the context in ways that enable the person to perform successfully; and we can provide supports that enable the person to be successful. The good news is that a number of approaches, practices, and mindsets in special and inclusive education already conform with or are driven by person-environment fit models of disability, beginning with a strengths-based approach.

Strengths-Based Approaches

Person-environment fit models require that we begin by considering a student's strengths, abilities, and competencies. Historically, deficits and remedial approaches began with just the opposite: a determination of the student's deficits. Strengths-based approaches take, as a starting point, the assumptions of social models of disability and then translate them into approaches to support, educate, or enable students with disabilities to function successfully in typical contexts. There is considerable power in beginning with a student's strengths, among them that it can enhance a positive relationship between the student and the teacher.

The social models and strengths-based approaches lead to a support focus in inclusive education. Deficit-based models led to the development of "programs" that were predicated on the type or severity of the person's disability. Within person-environment fit models and strengths-based approaches, we are interested in identifying what supports a student needs, where supports are defined as anything that can enhance functioning and self-determination to enable a person to be successful. We return to this later in this book when we discuss personalizable education and student agency.

Schoolwide Approaches

There is an increasing recognition that the focus for inclusive education should be at the school level rather than simply at the classroom level. Schools are com-

munities that involve an array of students with differing experiences, abilities, and interests. The prevalent schoolwide approach in place now involves multitiered systems of supports (MTSS). An MTSS framework emerged from the combination of "tiered" interventions to address the needs of students who were struggling, academically, referred to as "response to intervention" (Fuchs et al., 2010), and to address students who were struggling behaviorally, through school-wide positive behavior support (Horner & Sugai, 2015).

In an MTSS model, all students receive high-quality, evidence-based, and universally designed instruction, referred to as tier 1 instruction, taking into consideration their linguistic and cultural backgrounds, abilities, and other areas of learning support needs. Historically, if students were struggling to learn, they would be "referred" to special education, which too often meant going somewhere other than the general education classroom for either part or all of the day. In MTSS models, consistent with person-environment fit models, what changes when students are struggling is the type, intensity, or duration of instruction. Thus, some students may receive what is referred to as tier 2 instruction, which changes the instruction in some way but not where the

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student receives that instruction. Finally, a few students may need more individualized, intensive supports to succeed, referred to as tier 3 supports. But it is important to understand that, throughout this process, as students move to more intensive levels (tiers) of support, they do not need to be removed from general education classes—again, what changes is the type, intensity, or duration of instruction.

The U.S. Department of Education's Office of Special Education Programs funded a national K–8 technical assistance center at the University of Kansas called the Schoolwide Integrated Framework for Transformation (SWIFT) Center, that emphasizes state- and district-level commitment to the education of all learners within a multitiered approach. The SWIFT model of MTSS emphasizes research-based, data-driven decisions to determine the type and intensity of instructional supports needed by students with disabilities. The model presumes that all students are involved in high-quality instruction and that data are available from all students to inform instructional decisions (McCart et al., 2014). A future book in the Norton Inclusive Education for Students With Disabilities

series will focus on the SWIFT model and provide more detail about schoolwide approaches to promote inclusive education.

Universal Design for Learning

A second illustration of how person-environment fit models of disability can impact practice in education involves the application of universal design for learning (UDL). Orkwis and McLane (1998, p. 9) defined UDL as "the design of instructional materials and activities that allows the learning goals to be achievable by individuals with wide differences in their abilities to see, hear, speak, move, read, write, understand English, attend, organize, engage, and remember." First, then, UDL is about the design of instructional materials and is a form of supplementary aids and services. Such materials are designed in ways that allow all students to benefit from the material. Think of it this way: If a student had difficulty reading and is in a sixth-grade language arts class where the rest of the class is reading one of the myriad young adult novels taught in that grade, simply providing a copy of the print book will be of little use to that student. We can, however, adjust how the content of that book is represented—say, provide it in digital format so the student can "read" the book using a digital talking book—that ensures the student can participate with other students in the class. Further, universally designed materials benefit a lot of students, not just students with disabilities.

Self-Determination

The focus on transition in the education of learners receiving special education services emerged in the 1980s as the first wave of students served under the EHA began to exit school to more and less optimal outcomes: more in that outcomes like employment and community inclusion improved as a function of the access to education that the EHA provided; less in that those improvements were incremental and fell short of what had been hoped for (Wehmeyer & Webb, 2012). One issue identified as potentially contributing to these less positive than expected outcomes was that students were not actively engaged in the transition process. As a result, the U.S. Department of Education's Office of Special Education Programs began a number of initiatives to remedy this, including a focus in the reauthorization of the EHA, renamed IDEA, on transition and student involvement in transition planning (Wehmeyer & Ward, 1995).

From these initiatives emerged a focus on promoting the self-determination of young people receiving special education services. Over the past three decades, evidence has accumulated on the importance of enhanced self-determination to more positive school-related performance and transition-to-adulthood outcomes. Further, evidence-based interventions and assessments have been developed and implemented to enable teachers to promote student self-determination (Shogren & Wehmeyer, 2020). A focus on self-determination is important both for personalized education, as discussed in Chapter 1, and in the context of person-environment fit models and strengths-based approaches. In personalized education, students will have to become owners of their own learning; they will need to be able to identify what they do well and what they are interested in and use problem-solving and goal-setting skills along with this knowledge to chart their own path toward their future. In the fields of vocational and career guidance and counseling and, more recently, in the field of transition services within special education, professionals are

increasingly adopting a life design approach (described in Chapter 5), which empha-

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sizes students designing and constructing their own careers and lives (Wehmeyer et al., 2019). Acting in a self-determined manner is central to these processes.

Further, strengths-based approaches place the student at the center of all learning. As we head into an era of personalized learning, student self-determined learning becomes critical to positive outcomes. We return to this in greater detail in Chapter 5.

Strengths-Based Approaches and Inclusive Education

Throughout the books in this series we will explore and expand on ideas introduced in this chapter to understand such topics as schoolwide approaches, universal design, supports, and self-determination, so as to “broaden our reach” with regard to inclusive education. In Chapters 3 and 4 we do a deeper dive into what we know about effective practices in inclusive education and evidence in support of inclusive education. In the final chapter we return to some of the issues to look at where we go from here in inclusive education. For now, it is worth asking what a strengths-based approach predicated on social models of disability gets us as educators interested in inclusive education.

First, in Chapter 1 we discussed the implications for special education arising from the *Endrew F.* Supreme Court decision. As a field, we cannot achieve the standards of an appropriately ambitious education that is based on a student's potential for growth unless we jettison deficit-focused educational practices and embrace strengths-based approaches. How we understand disability does matter when it comes to what we do "about" disability. As we asked at the beginning of this chapter, why is it that, even though the EHA and IDEA emphasize educating students with disabilities with their nondisabled peers, we are still struggling, almost 50 years later, to achieve that vision? In part, it is because how we have conceptualized disability most of those 50 years viewed students as incapable and incompetent.

In the 1997 reauthorization of IDEA, language was introduced that required schools to ensure that all students receiving special education services were involved with and progressing in the general education curriculum. These so-called "access to the general education curriculum" mandates were introduced to align the education of learners receiving special education services with (at that time) prevailing standards-based reform efforts. Independent of how one feels about that era and its concomitant emphasis on standards, accountability, and testing, one useful thing as we move forward is that these actions were undertaken to raise expectations for students receiving special education services, expectations that historically had been low (Lee et al., 2008; Wehmeyer, 2006). Strengths-based approaches allow us to move forward and to presume that students with disabilities can be successful and that we can have high expectations for them.

These social-ecological or person-environment fit models emphasize that disability is part of, and not apart from, typical human functioning. The WHO ICF and the social and identity models emphasize participation as a critical element of understanding disability. That is, disability is best understood in these models by looking at the participation of people with disabilities in all aspects of life. It is, in fact, the impact of health, environmental, and personal factors on a person's activities and full participation that defines disability. If someone is fully participating, even with accommodations and supports, then under the ICF model, while the impairment may still be there, disability is essentially irrelevant.

Thus, trends in personalized education and changing understandings of disability make inclusive education even more important, and in adopting strengths-based approaches and their related instructional models and strategies, we can

break the logjam with regard to fulfilling IDEA's expectations for the education of learners with disabilities.

Reflective Questions for Group Study

1. Compare and contrast social models and social-ecological models of disability and discuss the relative benefits of each.
2. How does a strengths-based approach to educating learners with disabilities change how education is structured?
3. Consider current special educational practices and discuss which such practices fit within social-ecological models of disability.